COMBATIVE PATIENT

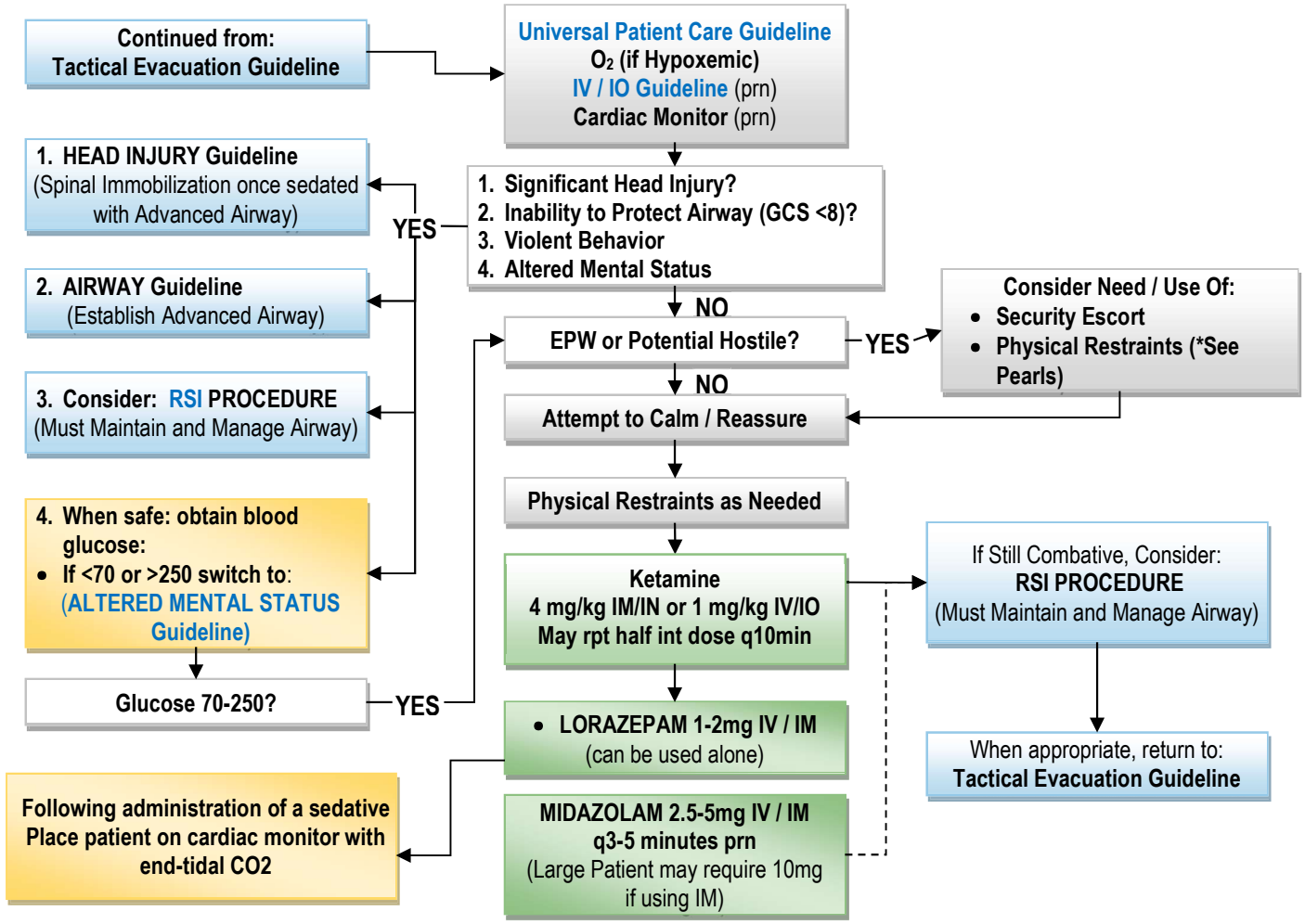
Signs and Symptoms:

- Bizarre Behavior
- Violent Activities
- Head Injuries / AMS
- Anxiety
- Tachycardia / Elevated BP

Differential Diagnosis:

- Head Trauma / Psychiatric Disorders
- Thyroid Dysfunction
- Hyper/Hypoglycemia
- Diabetic Ketoacidosis / Toxic Ingestion
- Environment (Hyper / Hypothermia)
- Hypoxia

Safety of Helicopter/Crew/other patients is first priority



Pearls:

- ***Physical restraints** such as tying down patient hands to prevent pulling lines, etc., should be limited to the least amount necessary to accomplish treatments / prevent injuries. **(Kerlex gauze can be a useful restraint)**
  - **Do not jeopardize the patient's airway!** – Avoid hog tying, lying prone in restraints, sandwiching between spine boards, etc.
  - **Check Vitals, SpO_2, Pulse and Cap Refill every 5 minutes.**
- Combative patients present a very real threat to the safety of themselves, the medic, and the aircrew during flight. For this reason, any patient with altered mental status and the potential for combativeness that would threaten aircrew safety or themselves should be prophylactically sedated / paralyzed and intubated for the flight.
- Use of sedative medications adds risk of decreasing respiratory drive and should be used with caution. However, meds should be titrated to adequate dosage to control patient. Be prepared for airway interventions/vomiting if used. Cardiac arrest in patients with excited delirium/extreme agitation following restraint is well documented. Capnography in addition to cardiac monitoring is essential.